BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show sequence, not equal lengths. Test timing follows the treatment, clinical findings and local pathway. The treating team owns booking, result review and action when a test is overdue.

Check	Before starting	Chemotherapy visits	Phase change / 12 weeks	Continuation / local care	Completion / follow-up
Blood tests	Baseline FBC, renal and liver profile	FBC, renal and liver profile before each treatment dose	Reassess before a new phase	Companion drugs / abnormal results determine additional tests	Follow persistent abnormalities and the next plan
Echo / cardiac assessment	Assess symptoms and cardiac risk; no fixed routine echo for this taxane alone.	Earlier assessment for symptoms or cardiac concern	Reassess at a clinical / exposure milestone	Assessment if clinically indicated.	At a change or completion, agree cardiac follow-up from treatment exposure, symptoms and results.
ECG / rhythm	Baseline / repeat if indicated by the cardiac assessment	Symptoms, interacting medicines or clinical indication	An ECG does not replace an echo	Repeat if symptoms or the cardiac plan require it	Agree follow-up if abnormal
Imaging / markers	Document baseline disease and purpose of scans	Review symptoms and response	Metastatic: scan if due	For metastatic breast disease, response assessment commonly every 2–4 months, adapted to sites, symptoms and treatment. Markers can supplement assessment; they do not replace imaging or alone determine a treatment change.	At a treatment change, document disease status and the next imaging plan. Tumour markers supplement imaging.
Bone health / DEXA	Assess endocrine treatment and fracture risk	Not a routine scan at each cycle	Baseline DEXA when indicated	DEXA before aromatase inhibitor / ovarian suppression when indicated; repeat according to fracture risk, results and endocrine plan.	Risk- and result-based follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00202 v8 · official source · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026 · EMA Herceptin product information

THE WHOLE TREATMENT JOURNEY

Docetaxel 100 mg/m² · every 3 weeks: the whole pathway

Locally advanced or metastatic breast cancer

Docetaxel 100 mg/m² by IV infusion on Day 1 every 21 days, with dose changes when needed. The number of cycles follows benefit, tolerance and the agreed treatment plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before systemic treatment	Chemotherapy phase	Next phase / reassessment	Local / continuing treatment	Follow-up / change
Cancer medicines NCCP 00202	Confirm indication, organ function and access	Taxane ± HER2 treatment	Review tolerance and response	Continue / change according to benefit	Review outcome and next treatment
Surgery METASTATIC	Site-specific procedures if indicated	No automatic surgery during these cycles	Reassess local-treatment needs	Individual multidisciplinary plan	Local follow-up
Radiotherapy METASTATIC	Assess indication	Only when clinically indicated	Coordinate surgery and recovery	Radiation team agrees timing	Assess late effects / local follow-up
Blood tests NCCP 00202	Baseline FBC, renal, liver	FBC, renal and liver profile before each treatment dose	Review before transition	Continue according to active medicines	Follow abnormalities
Heart tests NCCP 00202 · SMPC · ESC	Risk-based cardiac assessment	Symptoms can prompt earlier tests	According to cardiac risk	No universal fixed-cycle echo	Agree post-treatment cardiac plan
Scans / markers EARLY · METASTATIC	Baseline staging as indicated	Clinical response / symptoms	Response scan if due	Response imaging if metastatic	Symptoms can trigger earlier tests
Endocrine / bone health EARLY · BONE	Check receptors / endocrine indication	Discuss the later plan	Start / continue when agreed	May overlap HER2 / RT if appropriate	Longer-term bone health and endocrine review

What each check is for

1 **Blood checks**

FBC, renal and liver profile before each treatment dose

Results help the team judge treatment safety.

Ask / act: Confirm the next test and who reviews it.

Sources: NCCP 00202

2 **Heart assessment**

No routine fixed-cycle echo or ECG is mandated for this regimen alone; assess symptoms and individual cardiac risk.

Some heart changes occur without symptoms.

Ask / act: Keep the appointment and report new symptoms between scans.

Sources: NCCP 00202 · SMPC · ESC

3 **Imaging and markers**

For metastatic breast disease, response assessment commonly every 2–4 months, adapted to sites, symptoms and treatment. Markers can supplement assessment; they do not replace imaging or alone determine a treatment change.

Each scan should answer a defined clinical question.

Ask / act: Ask when the result will be reviewed.

Sources: EARLY · METASTATIC

4 **Booking and handover**

At initiation, phase changes and completion

Expected tests can be missed without an owner.

Ask / act: Record booking owner, result reviewer, due window and escalation contact.

Sources: NCCP 00202

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-17 · Source reconciled; independent clinical, pharmacy and patient review pending · SACTCheck v0.84.0